SURGICAL PATHOLOGY:

PROCEDURE DATE: [redacted] RECEIVED DATE: [redacted] REPORT DATE: [redacted]

COPY TO:

IO Consultation
FS DIAGNOSIS: "Adenocarcinoma (reported to Dr. [redacted] positive patient ID)" by Dr. [redacted]

Pre-Op Diagnosis
Lung mass
Post-Op Diagnosis
Same as above
Clinical History
Nothing indicated
Gross Description:
Five parts

Container labeled [redacted] right lung lower lobe wedge" has a previously partially sectioned wedge-shaped portion of recognizable pulmonary parenchyma having intact staple lines as the margin of resection. The specimen as received weighs 8.5 grams and measures 6.0 x 4.5 x 1.6 cm. The pleura is slightly wrinkled and gray-tan to brown. Eccentrically noted is a 2.2 x 2.2 cm area of induration and slight umbilication of the pleura. Sectioning beneath this reveals a poorly defined eccentric 2.3 x 2.0 x 1.5 cm nodular lesion with a gritty gray-tan fibrotic cut surface. This grossly extends up to the pleura but not through it. In addition, this extends to within 0.1 cm of the nearest staple line margin; however, this does not grossly appear to be transected at the staple line margin. The remaining parenchyma is spongy mottled tan-brown without additional lesions. A portion submitted for frozen section is reported as "adenocarcinoma (reported to Dr. [redacted] positive patient ID)" by Dr. [redacted] Also received in the same container are three tissue cassettes each labeled [redacted] The frozen residue is submitted labeled A. Representative sections of the remaining tissue to include the entire remaining lesion are submitted labeled B-G.

Container labeled [redacted] level 7 lymph node" has two

[redacted]

nodular portions of gray-tan fleshy tissue, 1.1 x 0.5 x 0.2 cm and 1.4 x 1.0 x 0.3 cm. The specimen is entirely submitted in a single cassette.

Container labeled "[redacted] - right lung lower lobe" has a 178 gram portion of recognizable pulmonary parenchyma grossly consistent with right lower lobe. The specimen as received measures 16.0 x 9.9 x 3.3 cm and has as its margin of resection the bifurcation of the tertiary bronchi. Adjacent to the bronchus is a 1.0 x 0.9 x 0.3 cm gray-black fleshy nodule. The nodule has a fleshy mottled gray-black cut surface. The pleura is smooth to slightly wrinkled mottled tan-brown with minimal anthracotic streaking. Noted on the anterolateral aspect at the basal margin is a Y-shaped staple line over an area of 5.2 cm. This is grossly consistent with a staple line matching that of the wedge-shaped portion of pulmonary parenchyma described in part one of this case. The staple line is grossly intact. On sectioning in this region, the parenchyma is hemorrhagic and spongy without gross lesions. This area is noted at its nearest point 4.0 cm from the nearest bronchial margin. The remaining parenchyma is spongy and tan-gray to pink with slight congestion; however, no discrete gross lesions are identified. Representative sections are submitted labeled as follows: A - shaved bronchial margin and bisected peribronchial nodule; B-F - entire shaved area of staple line parenchyma; G - remaining uninvolved parenchyma.

Container labeled "[redacted] - level 9 lymph node" has a 1.6 x 0.9 x 0.3 cm elongated portion of focally shaggy rubbery gray-brown tissue which is entirely submitted in a single cassette.

Container labeled [redacted] - level 4 lymph node right lung" has a 2.0 x 1.6 x 0.6 cm partially fragmented lobulated portion of focally shaggy gray-tan to brown fleshy tissue covered by yellow adipose tissue. This is received along with a 1.2 x 1.0 x 0.3 cm fragment of similar tissue. The larger portion is bisected and the specimen entirely submitted in a single cassette.

[redacted]

Microscopic Description:
The slides labeled [redacted] are examined. See diagnosis.

Final Diagnosis
Right lung, lower lobe (wedge biopsy):
Tumor characteristics:
Procedure: Wedge biopsy.
Specimen integrity: Intact.
Specimen laterality: Right.
Specimen site: Right lower lobe.
Tumor size: 2.3 cm in greatest diameter.
Tumor focality: Unifocal.
Histologic type: Adenocarcinoma with associated fibrosis (see comment).
Histologic grade: Moderately differentiated.
Visceral pleural invasion: Carcinoma extends up to but not through visceral pleura.
Lymphovascular space invasion: Foci highly suspicious for lymphovascular space invasion
identified.
Surgical margin status:
Carcinoma extends to within 1 mm of the staple line on this wedge

[redacted]

resection (see comment).
Other:
Emphysematous changes. [redacted]
Level 7 lymph node (biopsy):
Reactive sinus histiocytosis, lymphoid hyperplasia and anthracosilicosis, no metastatic
carcinoma identified within two lymph nodes (0/2). [redacted]
Right lung, lower lobe (lobectomy):
Specimen integrity: Intact, prior wedge biopsy resection noted (see comment).
Specimen laterality: Right.
Specimen site: Lower lobe.
Tumor characteristics: No residual carcinoma identified within lobectomy specimen, see
above diagnosis on wedge biopsy.
Bronchial margin: No carcinoma identified.
Three peribronchial lymph nodes demonstrating reactive sinus histiocytosis, mild lymphoid
hyperplasia, and anthracosilicosis, no metastatic carcinoma identified (0/3).
Area of staple line, prior wedge biopsy, demonstrating recent hemorrhage consistent with
surgery, no residual carcinoma identified. [redacted]
Emphysematous changes. [redacted]
Level 9 lymph node, right lung, specimen four (biopsy):
Reactive sinus histiocytosis, lymphoid hyperplasia and anthracosilicosis, no metastatic
carcinoma identified within one lymph node (0/1). [redacted]
Level 4 lymph node, right lung, specimen five (biopsy):
Fragmented lymph nodes demonstrating reactive sinus histiocytosis, lymphoid hyperplasia,
and anthracosilicosis, no metastatic carcinoma identified (see comment). [redacted]

pTN stage: pT1b N0

[redacted]

Comments
This is staged as a pT1b N0 carcinoma based on a size of 2.3 cm in greatest diameter and negative lymph nodes. The final stage should be performed in conjunction with the clinical history. The carcinoma is a moderately differentiated adenocarcinoma with areas of fibrosis suggestive of a so-called scar carcinoma. Although the overall histology is consistent with a lung primary, the possibility of metastasis should be ruled out clinically. The carcinoma extends up to but not into or through the visceral pleura. Clinical correlation and follow up is recommended.

The frozen section diagnosis is confirmed.

Specimen five was received fragmented and, therefore, an accurate node count cannot be performed.

At the request of the undersigned pathologist, these slides have been additionally reviewed by Dr. [redacted] who concurs with the diagnosis.

This test has been finalized at the [redacted]

REVISION:

[redacted]

The case was revised to add the correct CPT codes. There is no change in the original diagnosis.

REVISION DATE: [redacted]

<Sign Out Dr. Signature>

[redacted]